

Colon & Colorectal Cancer “ Clinical Trial & Treatment Timeline

Alert Window: Last 30 Days · Generated: 2026-03-11 21:04:17

Clinical Summary

Recent developments in colorectal cancer research highlight several new clinical trials targeting advanced stages of the disease. Notably, trials are investigating innovative treatment combinations, such as Tegavivint for metastatic colorectal cancer and dual-target CAR-NK cells for biomarker-selected patients. These studies may significantly impact treatment decisions, particularly for those with unresectable or metastatic disease, by offering new therapeutic options. Moving forward, it will be important to monitor the outcomes of these trials, especially those focused on specific biomarkers and treatment regimens.

AI-generated from deterministic classification data · 2026-03-11 21:04:17

Situation Summary

Events Detected: 10
High Significance: 0
Top Category: New Trial
Primary Stage: Stage IV
Primary Target: General
Research Activity: Active Monitoring

Event Timeline

EVENT CLUSTER “ Safety and Efficacy of Tegavivint in Patients With Metastatic Colorectal Carcinoma

MEDIUM **STAGE IV** **GENERAL** **NEW TRIAL** **VERIFIED** **PUBLISHED MAR 11**

Mar 11 20:02 · immediate · clinicaltrials.gov

A new clinical trial is evaluating the safety and effectiveness of Tegavivint for patients with advanced metastatic colorectal cancer, potentially offering a new treatment option for those with this challenging stage of the disease.

1 source(s) reporting · cluster 1 of 10

EVENT CLUSTER " A Phase Clinical Trial of RC148 Plus Chemotherapy as 1L Therapy for Unresectable or Metastatic Colorectal Cancer

MEDIUM

STAGE IV

GENERAL

NEW TRIAL

VERIFIED

PUBLISHED MAR 10

Mar 11 20:02 Â· immediate Â· [clinicaltrials.gov](#)

A new clinical trial is investigating the combination of RC148 and chemotherapy as a first-line treatment for patients with unresectable or metastatic colorectal cancer, potentially offering a new option for improving outcomes in this advanced stage of the disease.

1 source(s) reporting Â· cluster 2 of 10

EVENT CLUSTER " Dual-Target CAR-NK Cells for Biomarker-Selected Advanced Colorectal Cancer

MEDIUM

STAGE IV

PD-1/PD-L1

NEW TRIAL

VERIFIED

PUBLISHED MAR 10

Mar 11 20:02 Â· immediate Â· [clinicaltrials.gov](#)

A new clinical trial is exploring the use of dual-target CAR-NK cells in patients with advanced colorectal cancer, potentially offering a promising treatment option for those whose tumors express specific biomarkers.

1 source(s) reporting Â· cluster 3 of 10

EVENT CLUSTER " Low-Dose Radiotherapy to Sensitize Pucotenlimab Plus CAPEOX for pMMR Locally Advanced Rectal Cancer

MEDIUM

STAGE III

GENERAL

NEW TRIAL

VERIFIED

PUBLISHED MAR 4

Mar 11 20:02 Â· immediate Â· [clinicaltrials.gov](#)

A new clinical trial is exploring the use of low-dose radiotherapy to enhance the effectiveness of the drug pucotenlimab combined with CAPEOX chemotherapy for patients with locally advanced rectal cancer, potentially offering a more effective treatment option.

1 source(s) reporting Â· cluster 4 of 10

EVENT CLUSTER " A Clinical Study of Iparomlimab and Tuvonralimab Combined With Bevacizumab and Alternating Triweekly CAPOX/mCAPIRI Regimen as First-line Treatment for Unresectable Advanced Colorectal Cancer

MEDIUM

STAGE IV

VEGF

NEW TRIAL

VERIFIED

PUBLISHED MAR 3

Mar 11 20:02 Â· immediate Â· [clinicaltrials.gov](#)

A new clinical trial is exploring a combination of Iparomlimab, Tuvonralimab, and Bevacizumab with a chemotherapy regimen for patients with unresectable advanced colorectal cancer, potentially offering a new first-line treatment option.

1 source(s) reporting Â· cluster 5 of 10

EVENT CLUSTER â€” Everolimus in CDK12-Deficient Metastatic Colorectal Cancer (EVER-RECODE)

MEDIUM

STAGE IV

GENERAL

NEW TRIAL

VERIFIED

PUBLISHED FEB 27

Mar 11 20:02 Â· immediate Â· [clinicaltrials.gov](#)

The EVER-RECODE trial is investigating the use of everolimus for patients with CDK12-deficient metastatic colorectal cancer, potentially offering a new treatment option for this specific group.

1 source(s) reporting Â· cluster 6 of 10

EVENT CLUSTER â€” Effect of Perioperative Lidocaine or High-Dose Dexamethasone on Immune Response in Colon Cancer Surgery (PILDI Study)

MEDIUM

STAGE III

GENERAL

NEW TRIAL

VERIFIED

PUBLISHED MAR 6

Mar 11 20:02 Â· immediate Â· [clinicaltrials.gov](#)

The PILDI Study is investigating whether using perioperative lidocaine or high-dose dexamethasone during colon cancer surgery can enhance the immune response, potentially improving recovery and outcomes for patients with Stage III colorectal cancer.

1 source(s) reporting Â· cluster 7 of 10

EVENT CLUSTER â€” FOLFOX Chemotherapy Combined With Fruquintinib and Serplulimab as First-Line Conversion Therapy for Initially Unresectable pMMR/MSS Colorectal Cancer

MEDIUM

STAGE IV

GENERAL

NEW TRIAL

VERIFIED

PUBLISHED MAR 3

Mar 11 20:02 Â· immediate Â· [clinicaltrials.gov](#)

A new clinical trial is exploring the combination of FOLFOX chemotherapy with Fruquintinib and Serplulimab as a potential first-line treatment for patients with initially unresectable colorectal cancer, which may improve options for those facing advanced disease.

1 source(s) reporting Â· cluster 8 of 10

EVENT CLUSTER â€” Neoadjuvant Chemoradiotherapy Plus Tislelizumab With or Without Probio-M9 in pMMR/MSS Locally Advanced Rectal Cancer

MEDIUM

STAGE III

GENERAL

NEW TRIAL

VERIFIED

PUBLISHED MAR 10

Mar 11 20:02 Â· immediate Â· [clinicaltrials.gov](#)

A new clinical trial is exploring the combination of neoadjuvant chemoradiotherapy with the immunotherapy drug Tislelizumab, with or without the addition of Probio-M9, for patients with locally advanced rectal cancer, potentially offering new treatment options to improve outcomes.

1 source(s) reporting Â· cluster 9 of 10

EVENT CLUSTER “ FOLFIRI and Bevacizumab With or Without Pelareorep for Second-Line Treatment of Metastatic RAS-Mutated, Microsatellite-Stable Colorectal Cancer

MEDIUM

STAGE IV

VEGF

NEW TRIAL

VERIFIED

PUBLISHED MAR 4

Mar 11 20:02 · immediate · clinicaltrials.gov

A new clinical trial is exploring whether adding Pelareorep to the standard treatment of FOLFIRI and Bevacizumab can improve outcomes for patients with advanced colorectal cancer that has specific genetic characteristics.

1 source(s) reporting · cluster 10 of 10

Appendix A – Patient-Friendly Summary

Plain-language overview for patients, caregivers, and family members

What's happening

Recently, there have been some exciting developments in the world of treatments for advanced colon and colorectal cancer. New clinical trials are starting that explore various combinations of drugs and therapies. For example, one trial is looking at a new treatment called Tegavivint for patients with metastatic colorectal cancer. Another trial is testing a combination of RC148 and chemotherapy for patients whose cancer cannot be surgically removed. There are also trials focusing on new ways to enhance the effectiveness of existing treatments, like using low-dose radiotherapy with other drugs for locally advanced rectal cancer.

What this means for you

For patients, these developments could mean more options for treatment. If you are facing advanced cancer, it might be helpful to ask your oncologist about these new trials and whether they could be right for you. It's important to discuss your specific situation and any potential benefits or risks. Remember to keep an eye on how you feel and any changes in your health. While the journey can be tough, these advancements show that researchers are working hard to find better ways to help patients like you.

AI-generated in plain language from clinical data · 2026-03-11 21:04:17

This is not medical advice. Always speak with your oncologist before making any treatment decisions.

Appendix B – Colon & Colorectal Cancer Treatment Reference

Incidence & Trends

~150,000 new US cases/year ~52,000 deaths/year 4th most common US cancer

▲ Early-Onset Alert: CRC rates in adults under 50 have risen ~2% per year since the 1990s Adults under 55 now account for ~20% of all new CRC diagnoses Cause under active investigation (diet, microbiome, obesity, sedentary lifestyle)

Current Screening Guidelines (2025)

USPSTF / ACS / ACG – Average Risk: Begin screening at age 45 (lowered from 50 in 2021)

Colonoscopy every 10 years FIT (fecal immunochemical test) annually Cologuard every 1-3 years CT colonography every 5 years

High Risk (family history / Lynch syndrome / IBD): Begin at age 40 or 10 years before youngest affected relative – whichever comes first Colonoscopy every 1-5 years depending on risk

Symptoms at any age: Rectal bleeding, change in bowel habits >4 weeks, unexplained weight loss, iron deficiency anemia – immediate evaluation regardless of age

Key Biomarkers – Test All mCRC at Diagnosis

RAS (KRAS/NRAS) BRAF V600E MSI/MMR status HER2 NTRK fusion TMB

Stage IV Targeted Options

KRAS G12C (~3-4%) – sotorasib + cetuximab adagrasib + cetuximab

BRAF V600E (~8-10%) – encorafenib + cetuximab ± binimetinib

MSI-H/dMMR (~5%) – pembrolizumab (1st line preferred) nivolumab ± ipilimumab

HER2 amplified (~2-3%) – trastuzumab + pertuzumab trastuzumab deruxtecan

RAS/BRAF WT – FOLFOX or FOLFIRI + cetuximab or panitumumab (left-sided preferred)

1st Line mCRC Standard of Care

FOLFOX or FOLFIRI ± bevacizumab (RAS mutant / right-sided)

FOLFOX or FOLFIRI + cetuximab or panitumumab (RAS/BRAF WT, left-sided)

Pembrolizumab monotherapy (MSI-H/dMMR)

Key Resources

ClinicalTrials.gov NCCN Guidelines (nccn.org) Colorectal Cancer Alliance (ccalliance.org) Fight Colorectal Cancer (fightcrc.org) ACS (cancer.org)

Colon & Colorectal Cancer Clinical Trial & Treatment Timeline

Alert classification fields are generated using deterministic rule-based analysis. Alert summaries are AI-generated from primary government sources including ClinicalTrials.gov, PubMed (NIH/NLM), and FDA.gov. This report aggregates publicly available data from U.S. government databases for situational awareness only. No article text is reproduced. This report is made available under a free-use license for patient advocacy organizations, oncology practices, and individual patients. You may share or distribute it freely. You may not modify it, remove attribution, or republish it as your own work. Authorship and all intellectual property rights are retained by Randy Hinton. This report is provided for informational purposes only and does not constitute medical advice. Always consult your oncologist or a qualified healthcare provider before making any treatment decisions.

© 2026 Randy Hinton. All rights reserved. · https://petroai1492.github.io/delta/timelines/coloncancer_timeline.pdf